SAMPLE ANSWER · DISTAL RADIUS FRACTURE · CASE 4

Sample 7-minute ISBAR: Tomasz

Tomasz Nowak · 55-year-old man · Outpatient hand therapy clinic, private practice · about 1023 spoken words (≈ 7.3 min at 140 words per minute)

I Introduction

79 words

"Hi, I'm the occupational therapy student who has been seeing Tomasz Nowak in hand therapy today. Tomasz is a 55-year-old chef and café owner with type 2 diabetes, whose left distal radius fracture came out of a cast last week.

"I need to discuss him before we start his programme, because he can't straighten the tip of his left thumb after a painless click three days ago, and he has a burn on his palm that he didn't feel."

S Situation

139 words

"Tomasz was referred to commence range of motion and strengthening. When I asked him to lift his thumb with his palm flat on the table, he couldn't, and when he opens his hand the tip of his thumb stays bent. I can straighten it passively, but he can't hold it there. He says it's been like this for three days, since he felt a painless 'click' lifting a full 20-litre stockpot.

"Under a plaster on his left palm, just below the thumb, there's a blister about the size of a 20-cent coin with reddened skin around it, from a pan handle he says he 'didn't feel'.

"He's been back on full shifts lifting pots and trays since the cast came off. He's now asking for strengthening exercises and a full-duties letter, because he has lunch service in 20 minutes."

B Background

168 words

"Tomasz is right-handed. He slipped on a wet kitchen floor seven weeks ago and sustained an undisplaced fracture of his non-dominant left wrist, managed in a cast for six weeks. The cast was removed at the fracture clinic last week with advice to 'avoid heavy lifting until review'. The referral notes satisfactory position on X-ray at six weeks. The therapy assistant recorded his grip strength at intake, which he described as 'not bad'.

"He has type 2 diabetes and says his GP 'keeps nagging' about his sugars. He lives with his wife, Ana, who manages the café's bookings and accounts. He has income protection forms due 'by Friday' and is focused on getting his grip back for Christmas catering.

"Gaps I'd need to clarify: whether his thumb movement was documented when the cast came off, whether he's had an X-ray since, his specific loading restrictions and when his review is booked, his recent diabetes control, and whether he has any known reduced sensation in his hands or feet."

A Assessment

316 words

"My main concern is the thumb. A sudden loss of active extension at the tip of the thumb, starting with a painless click under load, may indicate a rupture of the tendon that straightens the thumb tip, the extensor pollicis longus. This is a recognised complication of distal radius fractures, including undisplaced ones, typically weeks after injury, because the tendon runs

close to the fracture site. It won't recover with exercise, and surgical options are best considered promptly, so it needs review by the orthopaedic or hand surgeon rather than routine strengthening.

"Occupationally, losing active thumb extension affects releasing objects, pinching and stabilising things, which a chef relies on even with his support hand: holding a pan, using tongs, plating and steadying food for knife work.

"Second, the burn. A burn he didn't feel, in someone with diabetes, raises two questions: whether he has reduced protective sensation in his hand, and whether this wound may heal slowly or become infected, especially working under a glove in a hot kitchen. Working with heat and knives with possibly reduced sensation is a genuine safety risk.

"Third, loading. Full shifts lifting heavy pots go beyond the advice he was given, and the click happened under heavy load. Cast removal doesn't mean the wrist is ready for unrestricted loading.

"His grip reading being 'not bad' is misleading. Grip strength doesn't test thumb extension or sensation, and testing grip before confirming loading restrictions isn't ideal. The referral's 'strengthening' was written before this new problem appeared.

"The paperwork and the Christmas season genuinely matter, because his livelihood depends on them, but I can't write a full-duties letter or comment on his capacity until he's been reviewed, because the picture has changed.

"As a student I can't diagnose the tendon problem, but I think the findings are clear enough to hold strengthening and request prompt surgical review, along with review of the burn."

R Recommendation

321 words

"First, I won't start strengthening or progress loading today. I'd dress the burn according to the clinic's protocol and ask the surgeon whether they'd like the thumb protected in a splint while he waits.

"Second, I'd like your help to contact the fracture clinic or hand surgeon today to arrange prompt review, ideally within days rather than at a routine appointment, with a clear description of the findings and timeline. If needed, his GP could refer urgently.

"For the MDT: from the surgeon, I need assessment of the thumb, the management plan, and clear movement and loading restrictions for the wrist and thumb. His GP or practice nurse should review the burn given his diabetes, check his glucose control and assess sensation in his hands and feet. The hand therapist, who may be an OT or a physiotherapist, would then follow the surgeon's post-operative or conservative protocol.

"I'd explain all of this honestly to Tomasz, acknowledge the pressure of the Christmas season, and ask how we can keep the café running safely, perhaps by delegating heavy lifting and hot work to staff for now, with Ana's help in planning rosters.

"For OT assessment, my principal short-term assessment would be observed performance of key kitchen tasks within his restrictions, such as a simulated task carrying a light pan, using tongs and plating, noting thumb function, technique, pain and safety around heat. Longer term, I'd use the COPM to capture his priorities, likely running service, catering and managing the business, and reassess after surgical review and rehabilitation.

> "Intervention would include burn and sensory safety education, such as checking temperature with his right hand and using insulated gloves, work modification and education about loading. For paperwork, I'd provide factual functional information for his insurer and a work letter once the surgeon's plan and restrictions are known, not today. I'd follow up after his surgical review. The priority today is prompt surgical review, not strengthening."

Coaching notes *not spoken*

The immediate priority

Sudden, painless loss of active thumb-tip extension after a click under load, weeks after a distal radius fracture, needs prompt orthopaedic or hand surgeon review instead of strengthening, plus review of an unfelt burn in a man with diabetes.

Distractors in this case

- **"Commence ROM and strengthening"** — written before the new thumb finding; cast removal is not clearance for heavy loading.
- **Grip strength "not bad"** — grip does not test thumb extension or sensation.
- **Income protection forms and the full-duties letter** — capacity has changed; paperwork follows the surgeon's plan.
- **Christmas catering deadline** — a central goal for the COPM, not a reason to load an injured hand.

Missing information to clarify

- Thumb function documented at cast removal.
- Any X-ray since removal; specific loading restrictions and review date.
- Recent diabetes control and sensation in hands and feet.
- What home programme he was given.

Useful references

- American Academy of Orthopaedic Surgeons, *Management of Distal Radius Fractures: Evidence-Based Clinical Practice Guideline* (2020).
- Law M, et al. *Canadian Occupational Performance Measure* (COPM), Canadian Association of Occupational Therapists.

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